SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Panitumumab + modified FOLFOX-6

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

RAS wild-type metastatic colorectal cancer · NCCP 00447 · version 6b · 14-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Panitumumab + modified FOLFOX-6 is used for bowel cancer that has spread and has a confirmed wild-type RAS result. This NCCP regimen is a first-line option. Your team explains the aim, alternatives and how benefit will be reviewed.

Schedule
Day 1: panitumumab, oxaliplatin, folinic acid and fluorouracil, followed by a fluorouracil pump for about 46 hours; disconnection is arranged around Day 3. A cycle lasts 14 days. Treatment continues while it is helping and side effects remain acceptable.

1. GENERIC SACT RISKS / DISCUSSION POINTS

- ☐ Purpose, expected benefit and treatment plan explained
- ☐ Treatment may be delayed, modified or stopped because of toxicity
- ☐ Unexpected, rare or serious adverse effects can occur
- ☐ Supportive medicines and urgent contact advice discussed
- ☐ Pregnancy avoidance / contraception / fertility discussed where relevant
- ☐ Bone-marrow suppression / infection risk
- ☐ Anaemia / fatigue and low platelets / bleeding
- ☐ Nausea / vomiting and bowel disturbance
- ☐ Mouth soreness / mucositis and skin changes
- ☐ Allergic / infusion reaction and other organ-specific toxicity

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Panitumumab
EGFR-TARGETED ANTIBODY

- ☐ Rash, dry skin, cracks and sore nails
- ☐ Eye inflammation or corneal injury
- ☐ Low magnesium, calcium or potassium
- ☐ Diarrhoea, dehydration and kidney problems
- ☐ Infusion reaction; rare serious lung or skin effects

Oxaliplatin
PLATINUM CHEMOTHERAPY

- ☐ Cold-triggered tingling / throat discomfort
- ☐ Persistent sensory neuropathy, sometimes lasting after treatment
- ☐ Allergic / infusion reaction
- ☐ Low blood counts and infection
- ☐ Nausea, diarrhoea and fatigue
- ☐ Rare serious lung or blood complications

Folinic acid
CHEMOTHERAPY MODULATOR

- ☐ Enhances fluorouracil activity and toxicity
- ☐ Rare allergic reaction
- ☐ Diarrhoea / mouth soreness in combination

Fluorouracil
CHEMOTHERAPY

- ☐ Diarrhoea, dehydration and mouth ulcers
- ☐ Low blood counts / infection
- ☐ Hand-foot skin soreness
- ☐ Cardiotoxicity including chest pain / ischaemia
- ☐ Severe early toxicity including DPD-related toxicity
- ☐ Pump / central-line complications

Discuss RAS selection, skin/nail and eye effects, electrolyte monitoring for eight weeks after completion, infusion reactions and rare serious lung/skin toxicity. Confirm DPD testing, antiemetics, port/PICC care and pump connection/disconnection. Discuss cold-triggered symptoms and potentially lasting neuropathy.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00447 v6b.

SACTCheck v0.83.5
Developed by Dr Paul O'Brien